

ALBERTA DISABILITY INCOME REFORM SERIES

Document 9 of the AISH/ADAP Policy Analysis Series

THE INTERGENERATIONAL CONTRADICTION

How ADAP Guarantees the Dependency It Claims to Prevent

April 2026

Section 1: The Policy Premise

ADAP — the Alberta Disability Assistance Program — is the provincial government's replacement for AISH, effective July 1, 2026. Its stated premise is that disability income support should be designed to encourage and facilitate movement toward employment and greater independence. The framework assumes that with appropriate employment supports, skills building, and financial incentives, a meaningful portion of disabled Albertans can reduce their reliance on government assistance over time.

This premise is not inherently unreasonable as applied to some recipients. But it rests on a foundation the government has simultaneously dismantled: the systems that produce independent, functional disabled adults do not begin at age 18. They begin at birth.

THE CORE ARGUMENT

ADAP is premised on the idea that the current generation of disabled adults can move toward independence.

The same government is simultaneously failing every upstream system — education, children's disability services, pediatric healthcare — that would give the next generation of disabled children any realistic pathway to independence.

The government cannot claim to be building toward independence while guaranteeing dependency in the next generation through documented, systemic failure.

Section 2: Three Children — One Household — One Contradiction

The following profiles are drawn from the documented circumstances of three children in a single household in Alberta. All three share a confirmed rare chromosomal syndrome. All three carry an ASD Level 3 diagnosis — the most severe classification under the DSM-5. Their profiles are presented without identifying information. They are not hypothetical. They are documented.

CHILD PROFILE 1 — The Child the System Has Already Determined Cannot Work

DOCUMENTED PROFILE

- Rare chromosomal syndrome with confirmed genetic documentation.
- ASD Level 3 — requiring substantial support across all settings.
- Non-verbal or minimally verbal communication.
- Active behavioural profile: aggression, flight risk, crisis presentations.
- Cannot be safely supported by substitute educational staff — documented in writing by the school.
- Requires one-on-one funded support worker for school attendance.
- Has never attended school without specialized one-on-one support.

POLICY CONTRADICTION

- ADAP is premised on a pathway to employment. This child has no such pathway — not through lack of effort, not through lack of supports, but because the severity of documented disability makes independent, sustained employment functionally impossible.
- The government has, through the school's own documentation, already acknowledged this child cannot be safely managed without specialized individual support. It has simultaneously designed an adult income program that assumes employment is the goal.
- When this child turns 18, ADAP — or whatever replaces it — will be the program they are assessed against. The same framework being applied to their parent today will be applied to them. The same income reduction. The same removal of appeal rights. The same employment-first premise.
- The government is not building toward independence for this child. It is deferring the bill.

CHILD PROFILE 2 — The Child the System Has Written Off as a Problem for Later

DOCUMENTED PROFILE

- Rare chromosomal syndrome with confirmed genetic documentation.
- ASD Level 3 — requiring substantial support across all settings.
- School has acknowledged in writing that no substitute educational assistant can safely provide support.
- When his EA is unavailable, he cannot attend school. This occurs without advance notice.
- A school meeting documented that guest EA support was the operative plan — four days before the same school argued in writing that no guest EA could safely support him.
- Active ATPC complaint: 7 grounds, 32-entry evidence index. Filed against the school principal.

- Accountability gaps at the school level are documented, timestamped, and unresolved.

POLICY CONTRADICTION

- The school system has not built a sustainable support framework for this child. It has managed him reactively and inconsistently, produced contradictory written positions about his needs, and left accountability gaps that required a formal complaint process to address.
- This is not a child being supported toward future independence. This is a child being managed in the short term while the question of his long-term trajectory is deferred.
- The government has no mechanism to hold the school accountable for the downstream cost of that deferral. If this child reaches adulthood without adequate educational foundation or transition planning, he becomes a more dependent adult — not a more independent one.
- ADAP assumes that employment supports can bridge the gap between disability and independence. They cannot bridge a gap that the educational system spent a decade widening.

CHILD PROFILE 3 — The Child Whose Documented Needs Are Being Actively Reframed

DOCUMENTED PROFILE

- Rare chromosomal syndrome with confirmed genetic documentation.
- ASD Level 3 — requiring substantial support across all settings.
- Active cardiac monitoring protocol with a 911 directive in place at school.
- If she presents with cardiac symptoms at school, her parent must be reachable and able to respond immediately.
- Documented medical needs that require direct, reliable parental availability during school hours.
- Supports she currently has are being reframed — her documented needs characterized differently than the medical record supports.
- Support removals are being positioned as outcomes of 'progress' — without clinical grounding.

POLICY CONTRADICTION

- When a child's documented medical and developmental needs are administratively reframed as improvements, the consequence is not that the child needs less support. It is that the child receives less support — while the underlying need remains.
- This is cost reduction positioned as clinical determination. The government's own documentation of this pattern — across FSCD, PDD, and the school system — is available.
- A child whose supports are removed on the basis of reframed need is a child being set up for crisis. The cost of that crisis — medical, developmental, educational, or systemic — will be carried by the next government, the next program, and the next caseworker.
- This child is not being supported toward independence. Her documented needs are being minimized to reduce current program cost, while the downstream risk is transferred forward.

Section 3: The Structural Contradiction

These three children share more than a household and a diagnosis. They share a systemic position: they are all currently being failed by the government programs responsible for giving disabled children a foundation for adult life — while the government simultaneously designs an adult income program premised on those children eventually achieving employment and independence.

THE CONTRADICTION IN PLAIN LANGUAGE	
	The government is telling the current generation of disabled adults: you should be working toward independence.
	The government is simultaneously telling the next generation of disabled children — through its actions, not its words:
	• We cannot staff your classroom consistently.
	• We cannot provide timely children's disability services.
	• We cannot tell you when PDD will be available when you turn 18.
	• We will reframe your needs downward to reduce program cost.
	• We will cut the advocacy organizations that could help your family navigate the system.
	These two positions cannot coexist. The government is manufacturing the dependency it claims to be reducing.

3.1 The Pipeline the Government Has Built

Children with severe disabilities who do not receive adequate educational support do not develop greater independence. Children whose documented needs are reframed downward to reduce program cost do not receive fewer services because they need fewer — they receive fewer services while the underlying need compounds. Children who age out of FSCD into a PDD system with an unreported waitlist and 70% still not meeting their caseworker do not become independent adults. They become dependent adults in a system with fewer resources and no appeal rights.

The pipeline from inadequate childhood support to adult program dependency is not speculative. It is documented in the developmental literature and in Alberta's own downstream cost projections. The government is not preventing future reliance on disability income programs. It is guaranteeing it — and then designing that future program to provide less income and fewer protections than the current one.

STAGE	WHAT THE GOVERNMENT IS DOING
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Early childhood (FSCD)	3-year average wait for full services. 85% of families still without caseworker after partial services begin.
School age (Education system)	No consistent accountability framework for children too complex for standard supports. Contradictory documentation. Complaint processes as the only recourse.
Adolescence (Transition planning)	FSCD ends at 18. PDD application can begin at 16 — but service is not immediate. Waitlist duration unreported for 3 years.
Adulthood (PDD/AISH-ADAP)	70% of eligible adults have not met their PDD caseworker. AISH replaced by ADAP with reduced income, removed appeal rights, and employment-first framework applied regardless of capacity.
Throughout	Advocacy organizations that help families navigate these gaps had their funding cut in March 2026 — at the moment of highest demand.

Section 4: The Fiscal Argument

The government has framed the AISH-to-ADAP transition as a fiscal responsibility measure. The downstream costs of the systemic failures documented above are not being counted in that calculation.

COST ITEM	DOCUMENTED RANGE
Child welfare intervention (if family stability collapses)	\$80,000 to \$250,000+ per child per year in government care
Delayed educational milestones	Increased Special Education costs throughout K-12; documented as a downstream outcome of poverty and instability
Psychiatric crisis intervention (ED visit)	\$1,000 to \$3,000+ per visit; Alberta ED psychiatric visits up 47% prior to ADAP transition
Adult program dependency (PDD/ADAP)	Compounds existing program costs with reduced capacity for supported independence
Net saving from AISH-to-ADAP transition (before downstream costs)	\$27 million — after \$22 million in administration overhead
Alberta Heritage Fund growth (Q3 2025-26 alone)	\$383 million

THE FISCAL REALITY
The government is saving \$27 million net from the AISH-to-ADAP transition — before any downstream costs are counted.

The documented downstream costs of failing to support the next generation of disabled children adequately include child welfare intervention, educational remediation, psychiatric crisis response, and compounded adult program dependency — all of which cost more than the program it is replacing.

This is not a savings. It is a cost transfer — from the disability income budget to every other budget the government manages.

Section 5: The Policy Cannot Have It Both Ways

If the government's position is that disability income support should be redesigned around employment and independence as goals, then the government must also ensure that the systems responsible for building the capacity for independence are adequately resourced, consistently staffed, and held to account.

It has not done this. The same ministry that administers AISH, FSCD, and PDD has applied hiring freezes across all three simultaneously. The same minister responsible for ADAP is responsible for a children's disability services system with a three-year average wait and an unreported adult transition waitlist. The same budget that cuts \$49 million from AISH has produced nominal increases to FSCD and PDD that are below the population-plus-inflation benchmark — meaning real cuts, despite the numbers.

A government that is genuinely building toward independence for disabled Albertans does not simultaneously dismantle every system that makes independence possible for the next generation of disabled Albertans.

A government that does both — that mandates employment-readiness in the adult income program while failing education, children's disability services, transition planning, and the advocacy sector simultaneously — is not reducing dependency. It is producing it. It is simply deferring the cost to the next fiscal cycle, the next government, and the next generation.

CONCLUSION

The AISH-to-ADAP transition cannot be assessed in isolation.

It must be assessed against the full policy environment in which it operates: an environment in which the children of AISH recipients are currently being failed by the school system, waiting years for children's disability services, facing an undisclosed transition waitlist into adult disability programs, and watching the advocacy organizations that could help their families navigate these gaps have their funding cut.

The government cannot have it both ways.

It cannot mandate independence as a policy goal for disabled adults while guaranteeing dependency as a policy outcome for disabled children.

This is not a political argument. It is a structural one.

The contradiction is built into the policy. And the next generation will inherit the consequences.

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